

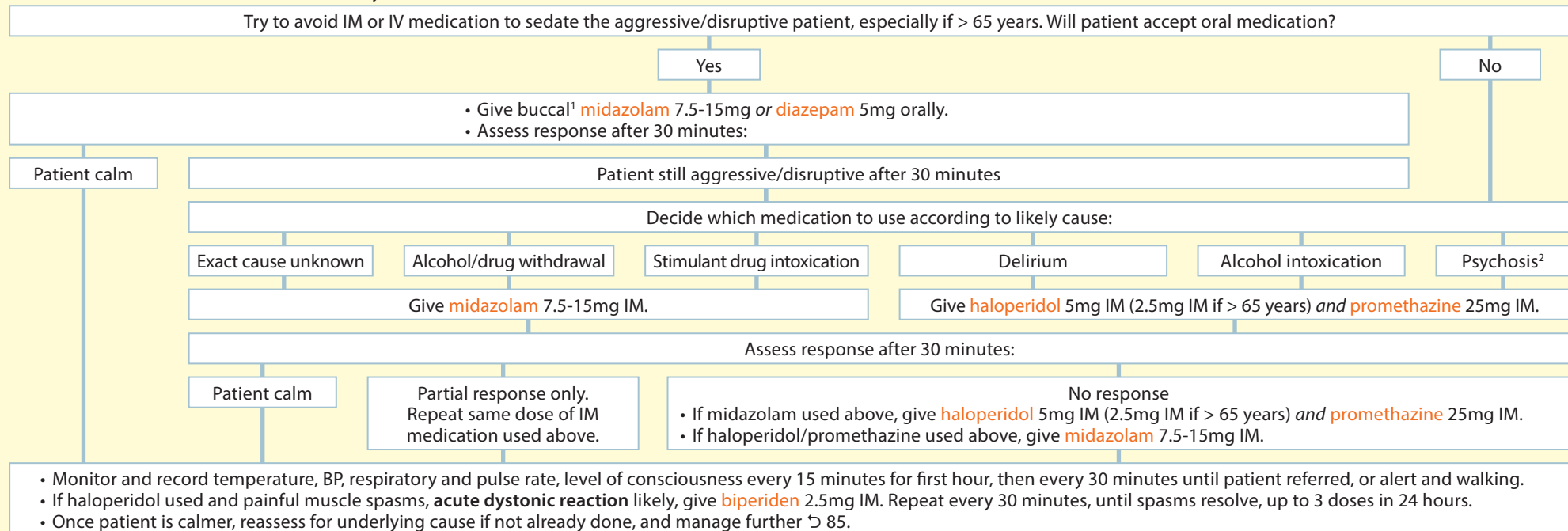
# AGGRESSIVE/DISRUPTIVE PATIENT

## Give urgent attention to the aggressive/disruptive patient with any of:

- Angry behaviour
- Loud, aggressive speech
- Challenging, insulting or provocative behaviour
- Frequently changing body position, pacing
- Tense posturing like gripping arm rails tightly, clenching fists
- Aggressive acts like pounding walls, throwing objects, hitting

### Management:

- Ensure the safety of yourself, the patient and those around you: ensure security personnel present, call police if needed. They should disarm patient if s/he has a weapon. Assess in a safe room with other staff. Ensure exit is not blocked.
- Try to verbally calm the patient:
  - Avoid direct eye contact, sudden movements and approaching patient from behind. Stand at least two arm's lengths away.
  - Use an honest, non-threatening manner. Avoid talking down to the patient, arguing or commanding him/her to calm down. Use a friendly gesture like offering a drink or food.
  - Listen to patient, identify his/her feelings and desires and offer choices. Take all threats seriously.
- Consider involuntary admission if signs of mental illness *and* refuses treatment or admission *and* a danger to self, others, own reputation or financial interest/property <sup>1</sup> 140.
- Restrain and/or sedate only if absolutely needed: imminent harm to self/others, disruption of important treatment, damage to environment, verbal attempts to calm patient failed.
  - If possible, before sedation: assess and manage possible causes of abnormal thoughts or behaviour <sup>2</sup> 85.
  - If restraints used, check restraint sites every 30 minutes.



- Refer the mentally ill aggressive patient same day to hospital<sup>3</sup>: document history, and time and dose of medication given. If emergency admission needed without patient consent, fill in **MHCA 01** form. If restraints used, complete **MHCA 48** form.

<sup>1</sup>Buccal: use IV formulation of midazolam, use syringe to draw up correct dose, remove needle and give midazolam between the cheek and gum. <sup>2</sup>Psychosis likely if patient not aware s/he acting abnormally and has ≥ 1 of: Hallucinations (seeing/ hearing things); Delusions (unusual/ bizarre beliefs); Disorganised speech or behaviour. <sup>3</sup>If delay in transport: try to move patient to most calm/quiet area and enlist help of a family member to monitor patient.